

SPEAK BEFORE THE REVEAL – CHEAT SHEET

Post-session reference guide - dictate before ending the scribe recording

Before you end the recording, say your reasoning aloud so that *your* formulation, risk stance and plan (not the AI's reconstruction) anchor the note. **Formulation, MSE, Risk and Plan** for all clinicians; **Key uncertainty, Differentials and Key phenomenology** additionally for trainees.

Formulation "My working formulation is..."

4 Ps Model

Predisposing - background vulnerabilities (genetics, early experience, temperament)

Precipitating - current triggers; what brought them in now

Perpetuating - maintaining factors (avoidance, relational patterns, environment)

Protective - strengths, supports, resilience, coping resources

Psychodynamic Lens

Core conflict - e.g. need for closeness vs. fear of rejection

Defence mechanisms - intellectualisation, humour, splitting, projection, avoidance

Attachment style - secure / anxious-preoccupied / dismissive-avoidant / disorganised

Repetition compulsion - re-enacting past relational patterns in therapy & life

CBT Snapshot

Thought-Feeling-Behaviour loop - identify the maintaining cycle

Core beliefs - e.g. "I'm unlovable," "The world is unsafe"

Cognitive distortions - catastrophising, black-and-white thinking, personalisation

Behavioural experiments - planned tests of feared predictions

Preserves: Integrative synthesis: why this person, why now **Guards against:** A plausible pseudo-formulation reverse-engineered from surface content

Mental State Examination "Their mental state showed..."

Appearance & Behaviour

Grooming & self-care, dress, apparent age, physical signs · psychomotor activity (retardation / agitation) · abnormal movements (tremor, tardive dyskinesia, tics, stereotypies) · eye contact · rapport · attitude (cooperative / guarded / hostile / evasive) · level of distress.

Speech

Rate, rhythm, volume, tone, quantity, latency, spontaneity, articulation - e.g. pressured, slowed, poverty of speech, dysarthria, dysprosody.

Mood & Affect

Mood (stated): Euthymic, depressed, elevated, anxious, irritable, dysphoric, labile.

Affect (observed): range, reactivity, congruence, quality (blunted / flat / restricted / full / expansive), appropriateness.

Thought Form & Content

Form: Linear & goal-directed, circumstantial, tangential, flight of ideas, loosening of associations, blocking, perseveration.

Content: Preoccupations, obsessions, overvalued ideas, delusions (persecutory / grandiose / nihilistic / somatic), ideas of reference, ruminations, suicidal / homicidal ideation.

Perception & Cognition

Perception: hallucinations (auditory / visual / olfactory / tactile), illusions, depersonalisation, derealisation.

Cognition: alertness, orientation, attention & concentration, short- and long-term memory, abstraction.

Insight & Judgement

Insight: awareness of illness, understanding of cause, attitude to treatment.

Judgement: decision-making, awareness of consequences, risk awareness, capacity to keep self-safe.

Preserves: Integration of observed and conversational data **Guards against:** An MSE limited to only what was verbalized, since the AI cannot see the patient

Risk "The risk I'm holding in mind is..."

Levels adapted from Rao et al 2017 (chronic vs acute)[1]. The crux is distinguishing acute change from chronic baseline: Applying acute / custodial responses to chronic risk is often iatrogenic.

Low chronic

Stable, longstanding baseline risk at a low level. Managed through ongoing therapeutic support, not acute intervention.

High chronic

Persistently elevated baseline (recurrent self-harm, high-lethality history) with no recent change. Contained within the therapeutic frame; treating as acute (admission / custodial care) tends to be iatrogenic.

New / emerging

A risk not previously part of the picture. A change warranting fresh assessment and closer monitoring; may herald developing acute risk.

Acute (acute-on-chronic)

Escalation above the person's baseline driven by acute precipitants (comorbid depression, substance use, recent discharge, acute stressor, perceived abandonment). The change that warrants active / protective response.

Assess: Chronic vs acute pattern (recent change) · Lethality · Chronic stressors · Current stressors · Protective factors.

Quality - Types of risk to consider: harm to self · harm to others · physical deterioration · mental-health deterioration · social / financial vulnerability.

Preserves: Dynamic risk formulation as a stance **Guards against:** Risk over-called from a metaphor, or under-called from non-obvious language

Plan "The agreed plan, and its reasoning, is..."

Consider: Action tied to the formulation (not a task list). **Psychotherapeutic:** therapeutic frame & relational stance · what is deliberately left open.

Pharmacological: medication rationale, including deliberate non-prescribing. **Assessment:** further examination/interview · investigations. **Next steps:** contingency / safety plan · follow-up interval and threshold to escalate · social support engagement

Preserves: Actions linked directly to the formulation **Guards against:** A superficial plan assembled only from spoken actions

Key uncertainty “I am least sure of...” trainees

Consider Competing formulations still open · missing collateral or information · what would change the plan · what to review next session.

Preserves: Calibrated doubt and a live differential **Guards against:** Declarative plans whose false confidence hides the gaps

Differential diagnosis “Differentials to examine include...” trainees

Consider Organic / medical mimics · substance-related · mood vs psychotic vs anxiety vs trauma vs personality · developmental (ASD / ADHD) in young people · what evidence would confirm or exclude each.

Preserves: Lateral diagnostic thinking **Guards against:** Generic, non-specific differentials

Key phenomenology “Key features supporting the formulation include...” trainees

Consider The specific symptoms / signs that anchor the leading hypothesis · their severity, duration and course · what remains a hypothesis versus what is established.

Preserves: Phenomenological reasoning and interpretive boundaries **Guards against:** Features of a hypothesis being recorded as an established diagnosis

Therapeutic Relationship Dynamics

Alliance & Engagement

Alliance strength - collaborative vs. resistant; degree of shared goals

Ruptures & repairs - tension, withdrawal, misunderstanding; how addressed

Boundary testing - lateness, cancellations, oversharing, prolonged silence

Dependence vs. autonomy - reassurance-seeking vs. self-directed ownership

Room affect - tension, warmth, humour, frustration, flatness, avoidance

Transference Patterns

How the patient relates to you through past relational templates

Parental - critical father / nurturing mother / absent caregiver

Peer / sibling - competition, camaraderie, rivalry, jealousy

Authority figure - expecting judgment, punishment, or validation

Rescuer / saviour - assumption you will fix everything; helplessness

Abandonment - hypervigilance to rejection, clinginess, pre-emptive withdrawal

Idealisation / devaluation - oscillating between you as perfect vs. useless

Countertransference Awareness

Your emotional responses as clinical data

Protective / rescuing - urge to shield; difficulty holding boundaries

Frustrated / irritated - feeling stuck, impatient, or provoked

Over-invested - ruminating outside sessions; boundary blur

Dismissive / numb - emotional disconnect, avoidance, boredom

Anxious / self-doubting - fear of saying the wrong thing; performance worry

Guilty / inadequate - sense of failing the patient; over-responsibility

Worked example

“**My working formulation is** that Sam’s presentation is a normative reaction to his parents’ separation, amplified by an anxious temperament and a school that has stopped scaffolding him - not a depressive disorder. **Their mental state showed** reactive affect, no psychomotor change, linear thought form, no perceptual disturbance. **The risk I’m holding in mind is** low chronic - no acute change, though I note new access to a sibling’s medication and am lowering my threshold to review. **The agreed plan, and its reasoning, is** to hold medication and begin family work first - the wait is the intervention - reviewing in two weeks, sooner if sleep worsens. **I am least sure** whether the irritability is depressive or an early bipolar profile; I’m watching, not committing.”

Blank prompt card *Glance and dictate*

Formulation: My working formulation is...

MSE: A+B · S · M+A · TF+TC · P+C · I+J

Risk: Level (low / high chronic · new-emerging · acute-on-chronic) + change from baseline · quality · lethality

Plan: The agreed plan, and its reasoning, is...

Key uncertainty (trainees): I am least sure of...

Differentials (trainees): Differentials to examine include...

Key phenomenology (trainees): Key features supporting the formulation include...

Companion to Table 1, “Speak Before the Reveal”. Verbalising these steps adds ~30–60 seconds and keeps the reasoning in active generation before the transition to passive editing.

- [1] S. Rao, J.H. Broadbear, K. Thompson, A. Correia, M. Preston, P. Katz, R. Trett, Evaluation of a novel risk assessment method for self-harm associated with Borderline Personality Disorder, *Australas Psychiatry* 25 (2017) 460–465. <https://doi.org/10.1177/1039856217707390>.